



POLICY

Multidisciplinary Team Clinical Reviews

Scope (Staff):	All Clinical and Administrative Staff
Scope (Area):	Child and Adolescent Mental Health Service (CAMHS)

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

All children / young person's and families receiving services from CAMHS must receive timely and regular Multidisciplinary Team (MDT) clinical reviews of their care a minimum, every three (3) months. This is in addition to individual assessments that form the basis of treatment, care and recovery planning (refer to [CP Standard - Care Planning](#)).

- This policy outlines the minimum requirements that all CAMHS teams must carry out when conducting an MDT clinical review. See individual directorate/team policy documents for any agreed upon additions and team processes.

Risk

Failure to conduct regular MDT clinical reviews may have the potential to impact upon clinical and organisational risk .

Background

The National Safety and Quality Health Service Standards require that services have formal processes to support and sustain multidisciplinary care teams, including current multidisciplinary treatment, care and recovery plans for children accessing the service.

MDT clinical reviews ensure that CAMHS clinical staff maintain high clinical standards in the processes of assessment, development and delivery of management, treatment and discharge plans for all children and families.

MDT clinical reviews are primarily a formal process to monitor treatment and progress. MDT clinical reviews also provide an opportunity to identify any barriers to discharge early in the episode of care, and plan interventions to address these barriers.

Definitions

Multidisciplinary Team (MDT) clinical review: A regular formal review process by a multidisciplinary team who assess and evaluate planned or provided treatment and directs changes in treatment to best accommodate the child and family needs. The discussion includes client assessment, risks, management, goals, treatment and discharge plans which is to be reviewed, as a minimum, every three (3) months (refer to [CP Standard - Care Planning](#)).

Chair of the MDT Clinical Review (Chair): The Chair for MDT Clinical Review meetings is to be the Head of Service or a Psychiatrist.

Child: An infant, child, adolescent or young person under the age of 18 years.

Family: Any parent, legal guardian or identified carer of the child.

Clinical / Case Formulation: Theoretically-based explanation or conceptualisation of the information obtained from a clinical assessment.

Health of the Nation Outcome Scales – Children and Adolescents (HoNOSCA): measure of outcome for use in child and adolescent mental health services focussing on general health and social functioning.

National Outcomes and Casemix Collection (NOCC): comprises a set of outcomes and casemix measures that focus on gathering information about a consumer's clinical status and functioning at various points during their engagement with a mental health service. The NOCC comprises both clinician rated measures and consumer rated measures.

Psychiatric Services On-line Information System (PSOLIS): covers a wide range of triage and service event items that staff can choose from to record their activity. Alert reminders are set at 91 days.

Treating Psychiatrist: The Psychiatrist within the service who has been nominated as responsible for the oversight of the child's care plan.

Key points

- MDT clinical review meetings ensure that the child and family have input from different relevant disciplines/professions throughout assessment, management, treatment and discharge planning when accessing CAMHS services.
- National Outcomes and Casemix Collection (NOCC) data must be used to monitor the progress of the child, evaluate the effectiveness of treatments and thereby provide information that will assist decisions about clinical practices. The service must ensure that all information is current.
- Clinical guidelines available from the CAMHS intranet ([Clinical Guidelines](#)) are available to assist reviews.
- Risks should be reviewed and updated including adding alerts to Psychiatric Services Online Information System (PSOLIS) or webPSOLIS .

MDT clinical review timing

- **Scheduled** MDT clinical reviews must occur:
 - after the completion of the CAMHS initial assessment. Noting the completion of the assessment document is not to delay the initial review
 - at a minimum every three months from the date of client admission.
 - Ward 5A: Additionally, within the first 48-72 hours of admission to the ward and at least weekly thereafter; AND
 - at discharge.
- **Ad-hoc reviews:** In addition to scheduled reviews, cases may be presented for ad-hoc (unscheduled) MDT clinical reviews. Reasons for this may include (but are not limited to) instances when:
 - The child or family:
 - requests a review;
 - declines treatment and support; or
 - is showing signs of non-engagement with the service; or
 - The child:
 - is at significant risk of injury to themselves or another person;

- receives involuntary treatment or is removed from an involuntary order;
- is transferred between CAMHS' teams;
- presents with an identified significant change in their circumstances which might impact upon risk;
- is observed through monitoring of their outcome measures (child and clinician rated instruments) to be deteriorating; or
- is or has been missing from hospital or day program.

Note that the occurrence of an ad-hoc review does not change the required timing for scheduled clinical reviews.

MDT clinical review participants

The MDT clinical review meeting must be chaired by the Head of Service or Psychiatrist;

The MDT clinical review should:

- be attended wherever possible by the child's treating Psychiatrist or delegate (Registrar or other senior clinician);
- involve clinicians drawn from at least three (3) disciplines or specialties;
 - Multisystemic Therapy (MST): the meeting should involve the maximum number of available disciplines involved in the care.
 - Ward 5A: For children who are likely to re-present to Perth Children's Hospital (PCH) Emergency Department (ED), invite a clinical representative from CAMHS Crisis Connect to the review meeting prior to discharge.
- Engage a CAMHS Aboriginal Mental Health Worker (AMHW) for Aboriginal children and families (see the [Aboriginal Mental Health Service Guideline](#)). The AMHW can provide cultural input into the assessment and formulation, and ensure that the MDT delivers services that take into account the cultural and social diversity of Aboriginal young people. For further information see the [CAMHS Aboriginal Mental Health Service Guideline](#);
- where possible, include CAMHS Peer Support Workers. This fosters a holistic and inclusive approach to treatment planning, and enhances engagement and support for the child and families / carers. Peer Support Workers can contribute valuable insights based on their lived experiences with mental health challenges and recovery, to enrich and bolster the treatment planning procedure.
- If the family attends the MDT clinical review, provide appropriate assistance and reasonable adjustments for those who do not speak, read or write English or who have communication difficulties. This includes hearing, oral or learning impairments. See [WA Health Language Services Policy](#).

MDT clinical review preparation and discussion

Agenda items and information are required prior to the MDT clinical review meeting. The Case Manager is responsible for gathering the relevant information. It is recommended that the information to be presented and discussed will¹ include (but not limited to) and follow a structured format (see Multidisciplinary Team Clinical Review Preparation Template Work Instruction):

- **Scheduled MDT Review:**

- Type of review;
- Reason for referral, presenting problems and findings on assessment;
- Relevant background, including family composition and context²:
 - Name/URMN/ Date of birth
 - child guardianship and residency (this may include foster carers noting that agencies e.g. Department for Child Protection and Family Support – (DCPFS) may not relay the information at MDT meetings to immediate carers)
 - history of significant events for the child and family,
 - school attendance, year
 - social relationship strengths and difficulties,
 - developmental concerns,
 - significant medical history,
 - previous mental health concerns and treatment,
 - issues associated with child's recovery and support systems;
- Gender pronouns, including options for recording the child's preferences on PSOLIS / webPSOLIS (e.g. via alerts) and notifying administrative staff of correct pronouns;
- NOCC data;
- Diagnosis and formulation, informing the treatment plan;
- Information pertaining to risk, including:
 - risk to self: suicidal and self-harm risk, ideation, behaviour and planning,
 - sexual risk,
 - risk of misadventure (ongoing drug and alcohol recording),

¹ As scheduled reviews are more regular on Ward 5A (weekly rather than 3-monthly), some of this information may not be required for some inpatient scheduled reviews.

² Information regarding the client's background should be discussed in more detail in the initial review and may be provided as a summary in subsequent scheduled reviews, noting any pertinent changes.

- child protection risks,
- risks to others,
- forensic history,
- current crisis management plan and alerts on PSOLIS/webPSOLIS;
- Current medication and information from periodical medication reviews, as clinically indicated (see [CAMHS Medication Safety Toolbox](#)).
- Physical health assessment status/medical care;
- Recovery planning, including the Treatment, Support and Discharge Plan (TSDP) and subsequent reviews of the TSDP (if one is already in existence), inclusion of anticipated length of stay, and therapy progress;
- The Crisis Management Plan (if in use);
- Other agencies involved and their roles and shared care arrangement status, if relevant ([see Guideline for Shared Care in CAMHS](#));
- Areas that the Case Manager / Care coordinator would like feedback on from the MDT;
- Discharge plan including predicted discharge date/most appropriate discharge services;
- Engagement in treatment and barriers to treatment;
- Agreed upon planned actions resulting from the MDT review (ensuring this is captured and recorded in client's medical record); and
- Any other relevant information e.g. phase of care (acute, functional gain, intensive extended or consolidating gains noting 'assessment only' applies to referral status only).
- **Ad hoc MDT review:**
 - Reason why the ad hoc review is required;
 - Agreed upon planned actions resulting from the MDT review;
 - Any combination of the above items, as deemed appropriate by the child's Case Manager or requested by the client, carers or other members of the MDT including changes to mental state (refer to [Appendix 1: Signs of Deterioration in a Person's Mental State](#))
 - Participants can attend in person or via tele/videoconference.

MDT clinical review responsibilities

Specific preparation responsibilities:

- If requested by the clinical team, **administrative staff** may be tasked with collating the agenda, ensuring the IT equipment is available, and preparing the room.

- The child's **Case Manager** (or delegate) is responsible for:
 - presenting the key clinical information and proposed formulation for review by the MDT.
 - The MDT review meeting should not be used as the primary forum to develop a formulation; it is expected that the Case Manager comes adequately prepared with a provisional formulation of the case.
 - inviting or consulting with external stakeholders;
 - explaining the MDT clinical review process to the child and family and exploring options for involving them in this process. This discussion must occur at the beginning of the episode of care;
 - Inviting and/or communicating with the client and parents/carers, as discussed and agreed with the child and family;
 - updating the TSDP, risk assessment, and any crisis / safety plans including assessment, diagnosis, formulation as per input from the clinical team;
 - implementing changes agreed upon in the MDT clinical review;
 - with written consent from the family, informing the child's General Practitioner (GP) and any other health professionals involved in the child's care of the outcome of the MDT review; and
 - identifying patients of concern during discharge reviews and taking action as per the [CAMHS Clinical Handover Guideline](#). Noting the family dynamics of each individual e.g. carers.
- The **treating Psychiatrist** is responsible for overseeing effective decision making about a client's care. If a treating Psychiatrist is not available to attend the MDT review at which their client is being discussed, a Registrar or other senior clinician must attend as a delegate. All decisions made at the review must be discussed with, and endorsed by, the treating Psychiatrist as soon as practicable.
- The **Chair** of a MDT clinical review meeting is the Head of Service or Psychiatrist (or their delegate) and is responsible for:
 - Delegation of the person to input the MDT meeting discussions into the medical record / patient notes and PSOLIS/ webPSOLIS
 - The conduct and timekeeping of the meeting;
 - Keeping the discussion focussed on the agenda; and
 - Adequate attendance of disciplines.

MDT clinical review documentation

Every CAMHS service must have a formal and consistent way of recording information presented and outcomes of the MDT clinical review. All information presented and discussed at the MDT clinical review must be documented in the medical record/patient notes **and** PSOLIS/webPSOLIS as delegated by the Chair.

Documents and forms to be completed include:

- A summary of the assessment or review must be completed during or immediately after the MDT clinical review; Noting PSOLIS documentation can be replicated in a MDT review - information can be added live throughout the meeting
 - Ward 5A: the summary must be completed during the MDT clinical review meeting to ensure that the information is available for iSoBAR handover.
- Following all **scheduled** MDT clinical reviews, the client's TSDP must be updated and signed by Case Manager (or delegate). The updated document must be discussed with, signed by, and a copy provided to the child / family as soon as practicable. Once signed by the child and family, a copy must be filed in the medical record. Any updates to management plans etc must be completed immediately following the review. This may not be necessary following an ad hoc MDT review.
- All MDT reviews must be documented in PSOLIS as clinical reviews within the Client Management wizard;
 - Clinical Reviews entered into PSOLIS for Community Mental Health Service Events are to be recorded as individual sessions.
 - The time spend discussing each client should be recorded separately where possible.
 - Where the recording of individual session duration is not possible, the total time for the discussion should be apportioned evenly between the number of clients discussed. For example, for a discussion of one hour, during which six clients were discussed, a ten-minute session duration would be recorded for each client.
- Where relevant, the client's Crisis Management Plan and any alerts on PSOLIS / webPSOLIS must be reviewed and updated as per the outcome of the MDT clinical review
 - See the Department of Health [PSOLIS Alerts Process](#) and [webPSOLIS infrastructure document](#), and the [CAMHS Risk Assessment and Management Policy](#) for further guidance regarding Crisis Management Plans and PSOLIS / webPSOLIS alerts.
 - [CAMHS Entry of Activity into PSOLIS Work Instruction](#)
 - For Ward 5A, also see the [PSOLIS alerts](#) work instruction.
- All MDT clinical reviews must be logged as service events in PSOLIS. The service event must list the review type (initial, scheduled, ad hoc deactivation etc.), and a list of attendees;

If the assessment has not taken place as per the MDT clinical review timing section above, the explanation must be documented in patient notes.

Communication with the child, family and referrer

All CAMHS children / young adults and families / carers are encouraged to partner with clinicians to plan, communicate, set goals, and make decisions about their current and future care. Therefore, the child and family should be aware of and kept involved in the MDT clinical review process. The Case Manager is responsible for explaining the MDT clinical review process to the child and family and exploring options for involving them in this process. This discussion must occur at the beginning of the episode of care.

All families must be given an opportunity to identify issues or concerns they would like raised in the MDT clinical review, and must receive feedback regarding any such concerns raised. Additional options for involving the child and family in the MDT process may include:

- Inviting the family to attend the MDT clinical review meeting. This must only occur where clinically appropriate, giving consideration to the information being shared at the meeting, and where the service has capacity to facilitate family attendance. Where it is agreed that a family will attend an MDT clinical review, sufficient notice must be provided to the family;
- Inviting the family to attend a “family meeting” following the MDT clinical review, where the discussion and outcomes of the MDT clinical review can be shared by the clinical team;
- Communication by the child’s Case Manager (or delegate) of any key decisions arising from the MDT clinical review (e.g. treatment changes, discharge plan, medication or any other changes to the TSDP). This may be a separate discussion, or occur as part of the child and family’s ongoing involvement in the development of the TSDP.

Following all scheduled MDT reviews, a written update summarising all key decisions must be provided to the relevant referrer / GP, as per consent and shared care arrangements. A copy of the written communication must be filed in the medical record. The updated, signed TSDP must also be filed in the medical record as evidence that the outcomes of the MDT review have been shared with the child and family (see [CAMHS Shared Care Guideline](#)).

Following ad hoc reviews, communication with the child, family and referrer may be assessed as not appropriate or not required, dependent on the reason for review and what (if any) decision-making occurred. For example, it may not be appropriate to share the clinical review with the young person and family if this could increase risk (including risk to staff), or if there is confidential discussion regarding child protection concerns.

Training

Training around MDT clinical review forms part of the CAMHS clinical skills training and is also included in the CAMHS Orientation Handbook (see [MyLearning - \[CAMHS\] Orientation Event & Handbook \(health.wa.gov.au\)](#)).

Compliance

Compliance with this policy is monitored via the CAMHS documentation audit, which occurs as per the CAMHS audit schedule.

Related CAHS internal policies, procedures and guidelines
CAMHS Aboriginal Mental Health Service Guideline
CAMHS Clinical Assessment Policy
CAMHS Clinical Supervision Policy
CAMHS Clinical Guidelines
CAMHS Clinical Handover Procedure
CAMHS Entry of Activity into PSOLIS Work Instruction
CAMHS Medication Safety Toolbox
CAMHS Risk Assessment and Management Policy
CAMHS Shared Care Guideline
Ward 5A PSOLIS Alerts Work Instruction

References and related external legislation, policies, and guidelines
Chief Psychiatrist's Standards for Clinical Care
WA Health Clinical Handover Policy
National Safety and Quality Health Service Standards
National Standards in Mental Health Services - Australian Commission on Safety and Quality in Health Care
Charter of Mental Health Care Principles MHA 2014
Review of admission or referral to and the discharge and transfer practices of public mental health facilities/services in Western Australia (Stokes, 2012)
DoH PSOLIS Alerts Process
DoH webPSOLIS infrastructure document
Community Mental Health Patient Activity Data Business Rules July 2023
Gaskin C, Dagley G. Recognising Signs of Deterioration in a Person's Mental State. Sydney: ACSQHC; 2018

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Appendix 1: Signs of Deterioration in a Person's Mental State

Reported Change	Distress	Loss of Touch with Reality or Consequence of Behaviours	Loss of Function	Elevated Risk to Self, Others or Property
Descriptions				
A person, or someone who knows the person well, reports that her or his mental state is changing for the worse.	A person, or someone involved in her or his care, shows signs of distress, which are evident through observation and conversation.	A person is losing touch with reality or the consequences of her or his behaviour.	A person is losing her or his ability to think clearly, communicate, or engage in regular activities.	A person's actions indicate an increased risk to self, others, or property.
Clusters of Signs				
Self-initiated requests for assistance ^a Requests for treatment from healthcare professionals or those close to the person ^a Self-reported negative or inflated sense of self ^a Self-reported uncontrollable thought processes Self-reported negative emotions ^{a b}	Uncharacteristic facial expressions ^a Physiological/medical deterioration ^a Negative themes in conversations ^a Apparent distress of self or others	Indications of experiencing delusions ^a Indications of experiencing hallucinations ^a Unusual self-presentation Unusual ways of behaving ^b Appearing confused during conversations	Unusual movement patterns Loss of skills Poor daily self-care Reduction in regular activities ^b Difficulty participating in conversations ^{a b} Unusual speech during conversations ^a Seemingly impaired memory Apparent difficulty with thinking about things in different ways ^a	Increases in the use of restrictive practices Reduced safety of self Reduced safety of others Reduced safety of property Disengaging from treatment ^a Unresponsiveness to treatment ^a